

HOW TO OPEN A GROUP HOME BUSINESS in California

Family homes & Adult Residential Facilities through the regional center system — serving adults with developmental disabilities.

A complete, plain-language, step-by-step startup guide — written so you can actually DO each step.

Figures current for 2026 · Every contact, rate & fee should be re-verified before you rely on it.

The California model — read this first

“Group home” in California means providing residential support to adults with intellectual and developmental disabilities, and the state funds it through the regional center system created by the Lanterman Act. California does NOT run developmental-disability services out of one state office. The Department of Developmental Services (DDS) funds and oversees, but the day-to-day work runs through 21 nonprofit REGIONAL CENTERS. Each regional center serves a defined area, determines eligibility, assigns a service coordinator, writes each person's Individual Program Plan (IPP), and purchases the services in it. To get paid you become a VENDOR of a regional center — a process called vendorization — which is the center of gravity for everything you'll do. If you open a licensed facility, a second body enters the picture: Community Care Licensing (CCL), the division of the Department of Social Services that licenses Adult Residential Facilities under Title 22.

The two ways in. Path A — become a certified FAMILY HOME through a Family Home Agency (FHA). An adult lives with your family; the FHA recruits, trains, approves, and supports you. Family homes are NOT licensed by Community Care Licensing — DDS and the regional center set the standards. This is the small, host-home-style option. Path B — open a licensed ADULT RESIDENTIAL FACILITY (ARF), the group home most people picture. It's licensed by CCL under Title 22 AND vendored by a regional center at a service level, with paid staff. Bigger operation, more oversight, more revenue. This guide walks both, and is honest about which is realistic when.

The names you'll deal with. DDS = Department of Developmental Services (funds the system, sets the rates, enforces family-home standards). Regional center = your local nonprofit (one of 21) — eligibility, the service coordinator, the IPP, and vendorization all run through it. CCL = Community Care Licensing (part of CDSS), which licenses ARFs/group homes under Title 22 — family homes are not licensed. FHA = Family Home Agency, the vendored agency that certifies and supports family homes (Path A). Vendorization = the approval process that makes you a paid provider, governed by Title 17. IPP = the Individual Program Plan the regional center writes for each person.

How to use this guide

Read it once start to finish, then use it as a checklist and a phone list. It's written for someone who has never opened a home — every time we name a thing, we stop, break it down, and tell you exactly where to go and what to ask. It is general information, not legal, tax, or billing advice; requirements, rates, and codes change (California just reformed its residential rates in 2025), so verify each step with your regional center, Community Care Licensing, your local fire authority, and qualified local counsel (see Part 11) before you rely on it.

The journey at a glance

These milestones take you from idea to your first payment — whether you start as a family home or open a licensed facility.

1	Decide your path & set up the	Path A (family home) or Path B
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	business	(licensed ARF); entity, EIN, NPI, insurance (Part 3)
2	Learn the standards	Title 17 vendorization & family-home rules; Title 22 for an ARF (Parts 4–5)
3	(Path A) Partner with an FHA	The FHA certifies, trains, supports, and pays you (Part 4)
4	(Path B) Get certified as an Administrator	ARF Initial Certification Training, state exam, live-scan (Part 5)
5	(Path B) Get the facility licensed	Community Care Licensing, Title 22 — orientation, application, inspection (Part 5)
6	Get vendored by your regional center	Program Design, service level 2–7, 45-day decision (Part 6)
7	Ready the home	Fire clearance, sprinklers, capacity, zoning, resident rights (Part 6→7)
8	Staff & write your policies	Direct support professionals, RN oversight, documentation (Parts 7–8)
9	Get a person placed	Eligibility, IPP, the service coordinator's match (Part 9)
10	Deliver services & get paid	Bill the regional center monthly at your vendored rate (Part 10)

Rule of thumb. A certified family home (Path A) is the small, fast way in — no CCL license, closest to a host home. A licensed ARF (Path B) is the scalable business, but it's a months-long project with two approvals running in parallel: a Community Care Licensing license AND a regional-center vendorization. Many people start by working as a DSP in a vendored home to build the experience regional centers expect, then open their own. Start smaller than you think, and let your regional center be your guide.

Before you begin — a realistic picture

Two honest truths will save you time and money. First, this is a relationships-and-documentation business funded by the regional center system, not a real-estate play or a normal small business: your revenue depends on getting licensed (for an ARF) and vendored, and then on the regional center's service coordinators referring people to you — so your reputation with your regional center, Community Care Licensing, and the service coordinators is your most valuable asset. Second, the money arrives on a chain and on a delay: an ARF needs administrator certification, a property, a CCL license, and a vendorization (a 45-day decision once your application is complete) — often several months end to end — and you don't bill until a person is placed and their services are authorized in an IPP, so you need patience and a working-capital cushion to carry staff and setup costs through the ramp.

Here's how the rest of this guide is organized: Part 1 explains what the business is and why now; Part 2 explains who regulates and pays you (DDS, the regional center, CCL, and Medi-Cal); Part 3 sets up the business itself, with the EIN, NPI, bank account, and insurance broken all the way down; Part 4 walks Path A (the certified family home); Part 5 walks Path B (the licensed ARF — administrator certification and CCL licensing); Part 6 is vendorization — the key that unlocks payment; Part 7 covers the home and fire safety; Part 8 is staffing; the policies and records section and the person's journey follow; Part 10 is how you get paid; and Part 11 collects every phone number and script in one place. Read it once end to end, then keep it beside you as a checklist.

PART 1

What This Business Is

In short: *Providing residential support to adults with developmental disabilities through California's regional center system — either as a certified family home (an adult living with your family, supported by a Family Home Agency) or as a licensed Adult Residential Facility with paid staff. Paid by the regional center, after you're certified/vendored.*

A “group home” business in California is really a developmental-disabilities support business. You help an adult with an intellectual or developmental disability live successfully in the community — with daily support, supervision, skill-building, help with health and medications, and a real home life — rather than in an institution. California offers two very different ways to do this: a certified family home, where an adult lives with your family and a Family Home Agency supports the arrangement; or a licensed Adult Residential Facility (ARF), the staffed group home licensed by Community Care Licensing and vendored by a regional center at a service level (2 through 7) that reflects residents' needs and drives your rate.

Be clear-eyed about what kind of business this is. This is a human-services business built on relationships and documentation, funded through the regional center system — not a real-estate play. Your value is the quality and reliability of the support you provide and your ability to keep it compliant and well-documented, because that's what the regional center vendors and pays for, what CCL licenses (for an ARF), and what keeps people safe. Go in understanding that the care and the paperwork ARE the business.

Why this business — and why now

Demand for community-based services for people with developmental disabilities is high and durable: California, like every state, is moving people out of institutions and into the community, families overwhelmingly want a real home for their loved one, and the regional center system funds exactly this. Because each person is served by a regional center and their IPP identifies the setting and service level they need, a reliable, well-run provider who builds relationships with service coordinators can keep its home full. And you can start lean — as a certified family home supporting one person — and grow into a licensed facility as you learn the system.

Be honest about the two realities that trip up new providers. (1) Two approvals, not one: for an ARF you don't just get a state license and bill — you get LICENSED by Community Care Licensing AND VENDORED by a regional center before a dollar flows, and they're separate processes with separate applications. (2) Room and board is separate from your pay: the regional center pays for the SERVICE, not the person's rent and food, which are handled separately (usually from the person's SSI/personal income). Plan your business — and explain it to families — around both.

PART 2

Who Regulates You & Who Pays You

In short: DDS funds and sets the rates; your regional center determines eligibility, writes the IPP, vendors you, and pays your claims; Community Care Licensing licenses an ARF under Title 22; and Medi-Cal is the underlying funding. The service coordinator is your key relationship.

DDS — the Department of Developmental Services (funds and oversees)

DDS funds the system, sets the rates through the Vendor Rate Study models, and enforces family-home standards. It does not run services directly — that's the regional centers — but it's the source of the rate models you'll live by, and it runs the statewide info line and the regional-center lookup (833-421-0061 · info@dds.ca.gov · dds.ca.gov/rc).

Your regional center — eligibility, the IPP, and vendorization (your payer)

Your local regional center (one of 21 nonprofits under the Lanterman Act) is the center of gravity. It determines who is eligible, assigns each person a service coordinator, writes the Individual Program Plan (IPP) that identifies the service and level a person needs, vendors you (Title 17), and pays your claims. The service coordinator is the person who refers residents to you and confirms the match — treat that relationship as central.

Community Care Licensing (CCL) — licenses an ARF (Path B only)

For a licensed Adult Residential Facility, Community Care Licensing — part of the California Department of Social Services (CDSS) — issues your facility license under Title 22, runs the orientation, reviews your application and program plan, conducts background checks, and inspects the facility (including the fire clearance). A certified family home (Path A) is NOT licensed by CCL.

Medi-Cal — the underlying funding

California's developmental-services system is funded through a blend of state general fund and Medi-Cal (including Home and Community-Based Services). For you as a provider, the practical interface is the regional center — you bill the regional center at your vendored rate rather than submitting Medi-Cal claims directly for residential services. Your NPI still matters for the system's billing and provider identification, so set it up correctly.

What this means for you: the path to payment runs through your regional center. For a family home the chain is FHA certification → the regional center pays the FHA, which pays you. For an ARF it's a CCL license PLUS a regional-center vendorization → the regional center authorizes and pays you monthly. Each link takes time; start them in the right order and in parallel where you can, and treat the service coordinators as key relationships, because they refer the people who fill your home.

PART 3

Form Your Business & Insure — Step by Step

In short: *This is where most first-timers get stuck. We break each piece all the way down — what it is, where to go, exactly what to do, and what to have in hand.*

3.1 Form your legal entity (an LLC)

An “entity” is the legal business — almost always a Limited Liability Company (LLC) — that holds your license or registration, your contracts, and your bank account, and that shields your personal assets if the business is ever sued. In California you form the LLC through the Secretary of State. Your entity, EIN, and NPI all have to be in place and consistent before licensing and vendorization, because they're checked along the way. You'll name a “registered agent” (a person or service with a physical in-state address who receives legal mail — this can be you) and adopt an Operating Agreement (the internal document that sets out who owns what and who can make decisions).

- 1. File your Articles of Organization with the California Secretary of State.** Online at bizfileOnline.sos.ca.gov — the LLC filing fee is \$70. Name the LLC and list a registered agent for service of process (this can be you).
- 2. File your Statement of Information within 90 days.** California LLCs must file an initial Statement of Information (Form LLC-12) within 90 days of formation, then every two years — it lists your managers, address, and agent.
- 3. Budget for the \$800 annual franchise tax.** Every California LLC owes the \$800 minimum annual franchise tax to the Franchise Tax Board to stay in good standing — plan for it as a fixed yearly cost.
- 4. Adopt an Operating Agreement.** The internal document that sets ownership and management — your bank, your license file, and your vendorization will reference your structure.
- 5. Confirm local business licenses.** Check what city or county business licenses apply where you'll operate.

No newspaper publication in California. California has no LLC publication requirement — forming the entity is a Secretary of State filing plus the initial Statement of Information and the \$800 annual franchise tax. Don't skip the Statement of Information: missing it draws penalties and can suspend your LLC.

3.2 Get your EIN — the free federal tax ID

An EIN (Employer Identification Number) is your business's federal tax number — think of it as a Social Security number for the company. It's free, takes about ten minutes, and you need it before you can open a bank account, run payroll, or enroll with any payer. Here's exactly how to get it:

- 1. Go to IRS.gov and open the EIN Assistant.** Search “Apply for an EIN online.” Apply directly on the IRS website — never pay a third-party site that charges for this free service.
- 2. Choose your entity and enter the responsible party.** Select “Limited Liability Company,” then give the responsible party's legal name and SSN or ITIN (usually you, the owner), plus your business name and address.
- 3. Finish in one sitting.** The online session times out after about 15 minutes of inactivity and can't be saved partway — have your details ready before you start.
- 4. Save your EIN and the CP-575 letter.** You get the EIN on the spot and can download the official CP-575 confirmation letter — save both; your bank and every payer will ask for them.

3.3 Get your NPI — the national provider ID

An NPI (National Provider Identifier) is a free, ten-digit ID number that health-care payers use to identify your organization. California's residential services are funded through the regional center and Medi-Cal, and your NPI is how the system identifies your organization for billing and provider records — you'll use it on your enrollment and vendorization paperwork. Your AGENCY needs a Type 2 (Organizational) NPI — which is different from the Type 1 (individual) NPI a single person would get.

- 1. Create an I&A account.** At nppes.cms.hhs.gov, set up an Identity & Access (I&A) account for whoever will manage the number — usually an owner.
- 2. Start a Type 2 (Organizational) application.** Log into NPPES and choose Type 2, not Type 1. Type 1 is for an individual person; your business is a Type 2 organization.
- 3. Enter your legal name, EIN, and address, and pick your taxonomy.** Choose the taxonomy that fits a developmental-disabilities / community-based residential provider — confirm the exact one your regional center and Medi-Cal want. The “taxonomy” is the code that says what kind of provider you are — confirm the exact one your Medicaid program wants before you submit.
- 4. Submit — it's free and quick.** It usually processes in about 10 business days. Save the NPI; you'll use it on every enrollment and claim.

3.4 Open a business bank account

Keep the business's money completely separate from your personal money — it protects your liability shield and makes payroll, taxes, and payer deposits clean. Open the account once your entity and EIN are set up.

What to bring to the bank:

- Your EIN confirmation (CP-575) from the IRS
- Your filed Articles of Organization / Certificate of Formation and the entity ID number
- Your signed Operating Agreement
- A Beneficial Ownership form (the bank collects details on anyone owning 25%+ plus a control person)
- A government photo ID, SSN, date of birth, and home address for each owner/signer
- An opening deposit (often around \$100)

Where to go: a bank with lots of California branches — Bank of America, Wells Fargo, Chase, Union Bank, or a strong local credit union. Home care is payroll-heavy — you pay caregivers weekly, but Medicaid and insurers pay you weeks later — so ask specifically about a business line of credit or invoice financing to bridge payroll before you scale.

3.5 Get your insurance — what you need, who sells it, and what to ask

A residential developmental-disabilities provider brings staff into a home and supports vulnerable adults, so you need coverage built for a human-services provider — and Community Care Licensing and your regional center will expect proof:

Coverage	What it protects against	Must-have?
General & Professional Liability	Injury or property damage, plus claims about the SUPPORT and supervision you provide	Yes — foundational
Abuse & Molestation	Allegations of abuse of a person you support — general liability EXCLUDES this	Yes — high exposure
Directors & Officers / Management	Regulatory, employment, and governance claims	Recommended for an agency
Non-Owned & Hired Auto	Staff driving their own cars to support or transport residents	Yes — critical
Workers' Compensation	Staff injuries — required in California with employees	Required in CA
Property	Coverage on the facility you own or lease	If you own/lease the home

Abuse & molestation is your highest exposure. Supporting vulnerable adults means abuse & molestation coverage — EXCLUDED by standard general liability — is essential, with a real limit, not a token sub-limit. Confirm it explicitly, because Community Care Licensing and your regional center care about it.

Real places to get it

Buy through an independent agent or broker who writes HUMAN-SERVICES / developmental-disabilities provider programs — this is a specialty niche. Carriers and programs active in DD/IDD and social-services work include NSM/Irwin Siegel Agency (a social-services specialist), Negley Associates, Philadelphia Insurance (PHLY), and CNA. Find a local California agent at trustedchoice.com and ask specifically for an Adult Residential Facility /

developmental-disabilities provider program. Confirm the carrier writes this in California before you rely on a quote.

What to ask the insurance agent

“Do you write Adult Residential Facilities / developmental-disabilities providers in California — which carriers, and what's the minimum premium?”

“Is abuse & molestation included, at what limit, and is it a sub-limit or a full limit?”

“Do you include non-owned & hired auto for staff driving the residents we serve?”

“What limits do Community Care Licensing and my regional center expect?”

“Can you set up California workers' comp for my direct-care staff?”

3.6 Keep your paperwork consistent — every step downstream checks it

One practical warning that saves California providers real time: your legal name, ownership, EIN, and NPI have to line up EXACTLY across every step that follows — your Community Care Licensing application, your regional-center vendorization (the online DDS Provider Directory), and any Medi-Cal records. The system means your information passes through more than one organization before a claim pays, and a mismatch at any handoff — a slightly different business name here, an old address there — is one of the most common and most frustrating causes of a stalled application or a rejected claim. Set the entity up once, correctly, and make every downstream document point at the same clean legal picture. It's worth an hour with your accountant or attorney up front to confirm the structure, because untangling a mismatch after you've applied costs far more than getting it consistent now. And keep your license, vendorization, Statement of Information, and franchise tax calendared for renewal — letting any one of them lapse stops your payments or your license even when the care is going perfectly.

The business foundation to have in hand before you apply for licensing or vendorization:

- Your formed California LLC (Articles of Organization filed, Statement of Information filed, \$800 franchise tax planned) and Operating Agreement
- Your EIN confirmation (CP-575) from the IRS
- Your Type 2 (organizational) NPI from NPPES
- A business bank account opened in the entity's exact legal name
- Bound insurance — general/professional liability, abuse & molestation, non-owned auto, and workers' comp
- A single, consistent legal name, ownership list, and address used identically on every document

With those six things in place and consistent, you're ready to build the licensing application (Part 5) and the vendorization application (Part 6) on a clean foundation — and you've removed the paperwork mismatches that stall more California applications than any substantive problem does.

PART 4

Path A — A Certified Family Home (through an FHA)

In short: *The small, personal, host-home-style option — with no facility license. An adult with a developmental disability lives with your family, and a Family Home Agency recruits, trains, approves, supports, and pays you. It's a partnership built on a good match.*

4.1 What a certified family home is

A certified family home means an adult with a developmental disability lives with your family and shares your everyday life. It's a partnership built on a good match, not a facility — and everyone in the household is part of the screening, so it's a family decision. Crucially, family homes are NOT licensed by Community Care Licensing; DDS and the regional center set the standards under Title 17 (sections 56075–56099), and a vendored Family Home Agency does the certification and support.

4.2 The steps

- 1. Make sure it fits your life.** A certified family home means an adult lives with your family and shares your everyday life. Everyone in the household is part of the screening — go in with eyes open about the good days and the hard ones.
- 2. Connect with a Family Home Agency (FHA).** You don't get licensed yourself — you partner with an FHA that's vendored by your regional center. The FHA recruits, trains, approves, and monitors family homes, and its social-service staff visit regularly. Ask your regional center for the FHAs in your area, and interview more than one — how they support and pay family homes varies.
- 3. Get approved and trained by the FHA.** The FHA completes your home approval and training under the DDS/regional-center standards in Title 17 (sections 56075–56099) — health and safety, individual rights, and a viable match. Because it's your private home, there's no Community Care Licensing license involved.
- 4. Welcome your family member.** The FHA and the person's regional-center service coordinator match you with someone whose needs and personality fit your home, with visits before the move. This match is the heart of the whole model.
- 5. Put your policies in writing.** Even in a family home, you'll keep clear documentation — medications, emergencies, incident reporting, and the rights of the person you support. PolicyReady's California manuals are built for exactly this.
- 6. Get supported, and get paid.** The FHA bills the regional center and pays you. That first payment is the moment it all clicks.

The difficulty-of-care tax note. *Because the person lives in your home, your family-home payments may qualify for the IRS “difficulty-of-care” exclusion (potentially non-taxable). Confirm with a tax professional — it can meaningfully change the economics of a family-home arrangement.*

What this means for you: Path A is the fastest, lowest-cost way in, and it's driven by the FHA's approval and a good match rather than a months-long licensing project. Many people start here — supporting one person, learning the regional center system, and building a track record — and grow into a licensed ARF (Path B) later. If you want to stay small and personal, a certified family home can be the whole business.

PART 5

Path B — A Licensed Adult Residential Facility (Group Home)

In short: *The bigger operation. Two approvals run in parallel: a Community Care Licensing license under Title 22 AND a regional-center vendorization (Part 6). This Part covers the business setup, the required experience, administrator certification, and the CCL license; Part 6 covers vendorization.*

5.1 Set up the business & get the experience

- **Form your entity and get your EIN.** Your LLC or corporation (Part 3) holds the license, the contracts, and the bank account.
- **Line up the required experience.** Regional centers expect the administrator to have direct experience in a vendored residential setting — many people work as a DSP in an ARF first to qualify. This experience is also how you'll actually know how to run the home.

5.2 Get your Certified Administrator certificate

An ARF must have a Certified Administrator, and the license lives with this role. You complete the ARF Initial Certification Training Program (a 35/40-hour course), pass the state exam, and clear a criminal-record background check through Community Care Licensing (live scan). The administrator must be at least 21 with a high school diploma or GED.

1. **Take the ARF Initial Certification Training Program.** The state-approved course (about 35/40 hours) that prepares you for the administrator exam.
2. **Pass the state administrator exam.** Sit and pass the Community Care Licensing certification exam for Adult Residential Facilities.
3. **Clear your live-scan background check.** Complete the criminal-record background check through Community Care Licensing — the administrator, and later your staff, must clear it.

5.3 Get your facility licensed (Community Care Licensing, Title 22)

Secure your home, then apply to Community Care Licensing for an Adult Residential Facility license under Title 22. This runs separately from — and in parallel with — your regional-center vendorization in Part 6.

1. **Attend the CCL orientation.** Community Care Licensing requires a licensing orientation before you apply — it walks you through the application and the requirements.
2. **Submit the application and program plan.** File your ARF license application, including a program plan describing how you'll operate and who you'll serve.
3. **Pass the background checks.** The administrator and staff clear Community Care Licensing background checks (live scan).
4. **Pass the fire clearance and licensing inspection.** Your facility must obtain a fire clearance from the local fire authority (or State Fire Marshal) and pass the CCL physical-plant inspection (see Part 7).

Two approvals, in parallel. *An ARF needs BOTH a Community Care Licensing license (this Part) AND a regional-center vendorization (Part 6) before you can take residents and bill. They're separate processes with separate applications — the CCL program plan is not the same as the regional center's Program Design. Run them in parallel to save months, and keep your legal name, EIN, and NPI identical across both.*

What this means for you: Path B is a real, months-long project — administrator certification, securing the property, the CCL application and inspection, and the vendorization in Part 6 — but it's the scalable business, with paid staff and a rate driven by your service level. The operators who get through cleanly treat the Title 22 requirements and the vendorization standards as the blueprint for the whole operation, so building your policies, staffing, and home to the standard does double duty: it earns your license and vendorization AND it's how you'll actually run.

PART 6

Get Vendored by Your Regional Center

In short: Vendorization is the key that unlocks payment — you cannot bill a regional center until you're vendored. You apply online through the DDS Provider Directory, write a Program Design, and are assigned a service level (2–7) that drives your rate. The center decides within 45 calendar days.

6.1 How vendorization works

You cannot bill a regional center until you're vendored. You apply online through the DDS Provider Directory to the regional center for the area where your home is located. You'll write a Program Design — based on person-centered, normalization principles, and different from the CCL application — and be assigned a service level (2 through 7) that reflects the residents' needs and drives your rate. The regional center makes a vendorization decision within 45 calendar days and assigns your vendor number and service code (or issues a denial you can appeal). Once vendored, other regional centers can also use you — but keep your same vendor number. Vendorization is non-transferable and doesn't guarantee referrals (Title 17, sections 54300–54390).

6.2 The service codes

California uses numeric service codes (not HCPCS). After the 2025 rate reform, the residential ones that matter are:

Service code	What it is
Adult Residential Facility — owner-operated 905	The group home you run yourself; rate by service level (2–7)
Adult Residential Facility — staff-operated 915	Group home run by employed staff; rate by service level
Supported Living Services (SLS) 896	Support for a person living in their own home
Independent Living Services (ILS) 520	Skill-building for independent living

For a group home, your everyday codes are 905 (owner-operated) or 915 (staff-operated), with your service level (2–7) setting the rate. A Family Home Agency has its own vendored service code, and the FHA bills the regional center and pays the family home.

Service level drives your rate. Levels 2–7 reflect residents' needs and required staffing; higher levels mean more staff and consultant hours and a higher rate, and Level 7 is customized for extraordinary needs. Your service level is assigned at vendorization and is the single biggest driver of what you're paid — so understand where your intended resident population falls before you apply.

What this means for you: vendorization is a quality gate, not a form. The regional center wants evidence — a real Program Design, real staffing, real capacity — that you can safely support people at the service level you're seeking. Build your Program Design and your operation to the standard and you'll clear the 45-day review AND have the operation you need to actually run the home.

6.3 What to have ready before you apply for vendorization

The 45-day clock starts when your application is complete, so applications stall most often because something is missing, not because the regional center says no. Have these in hand before you start the DDS Provider Directory application:

- ☐ Your consistent legal entity — the same LLC name, EIN, and address you'll use with Community Care Licensing (a mismatch is the most common stall).
- ☐ Your CCL license or a clear status on where the ARF license stands — the two run in parallel, and the regional center will want to see the licensing picture.
- ☐ A written Program Design based on person-centered, normalization principles — this is NOT the same as the CCL program plan, so write it separately to the Title 17 standard.
- ☐ The service level you're seeking (2–7) and the resident population it fits — this drives your rate and your staffing model.
- ☐ Your Type 2 NPI, bound insurance (including abuse & molestation), and your policies and procedures.
- ☐ The regional center that covers your home's location — confirmed by ZIP at dds.ca.gov/rc (in LA, this is assigned by health district).

With those ready and consistent, you submit online through the DDS Provider Directory to the regional center for your area, the center reviews, and — within 45 calendar days — assigns your vendor number and service code or issues a denial you can appeal. Keep a copy of everything you submit; you'll reference the same clean legal picture when other regional centers use you under the same vendor number.

Two things to know about your vendor number. (1) *Vendorization is non-transferable — it belongs to your entity, not to a buyer, so you can't sell your business and hand over the vendor number; a new owner vendors anew (Title 17, §54300–54390).* (2) *Being vendored doesn't guarantee referrals — the regional center's service coordinators still choose which vendored home to refer a person to, based on the fit with the person's IPP and their choice. Both facts point the same way: getting vendored is the start, not the finish. Keep your openings and specialty in front of the coordinators, and if you're ever denied, you have a right to appeal the decision.*

One more practical note: once you're vendored, you agree to accept the DDS-established rate as payment in full for the covered service and to bill only for services actually provided. That agreement is part of being a vendor — build your budget around the published rate for your service level, not a number you hope to negotiate, and keep your documentation clean so that what you bill always matches what you delivered. Remember too that the 2025 rate reform changed how these numbers are set: rates now flow from standardized DDS rate models published per regional center, so the figure for your service level in your area is a lookup, not a guess. Pull the current rate model for your regional center before you sign a lease or commit to a staffing plan, because your service level and that published rate together decide whether your home pencils out.

PART 7

The Home — Fire Clearance, Sprinklers, Capacity & Zoning

In short: A certified family home (Path A) is a private residence — NOT licensed by CCL — held to DDS/regional-center standards. A licensed ARF (Path B) is a licensed facility with a fire clearance, a physical-plant inspection, and Title 22 requirements. Which path you choose changes what your house has to be.

7.1 How California handles the home

A certified family home (Path A) is a private residence — NOT licensed by Community Care Licensing — so it's held to DDS/regional-center health-and-safety standards, not the Title 22 facility rules. A licensed ARF (Path B) is a licensed facility with a fire clearance, a physical-plant inspection, and Title 22 requirements. In every case, the home must genuinely be the person's home — with privacy, resident rights, and full access to the community.

7.2 “Do I need fire sprinklers?” — the honest answer

For a licensed ARF, it depends on two things: your capacity and whether any residents are nonambulatory or bedridden. Every ARF must obtain a fire clearance from the local fire authority (or the State Fire Marshal), and that clearance sets the requirements. A small ARF (six or fewer beds) serving ambulatory residents — people who can get themselves out in an emergency — generally is treated like a house and usually doesn't need commercial sprinklers. Once you serve nonambulatory or bedridden residents, or go to a larger capacity, sprinklers and stricter fire protection typically come into play. Because it's driven by the fire code and your local fire authority, confirm your exact home with them BEFORE you buy or renovate. A certified family home (Path A) is a private residence and doesn't carry the facility fire-clearance burden.

7.3 The ARF home checklist (Title 22)

- ☐ A fire clearance from the local fire authority (or State Fire Marshal) — and you must tell CCL whether any resident is nonambulatory or bedridden.
- ☐ Smoke detectors and fire extinguishers, posted evacuation plans, and regular fire drills.
- ☐ Capacity licensed in tiers — 6 or fewer, 7–15, 16–49, 50+ — with a 6-or-fewer home treated much like a single-family residence.
- ☐ A safe, clean physical plant: sound construction, working utilities, adequate bedrooms (no overcrowding), and accessibility appropriate to residents' needs.
- ☐ Stricter fire-clearance and building requirements for nonambulatory residents — plan your resident type and capacity before you sign on a property.

7.4 Zoning — California protects small homes

- **Six or fewer is a residential use.** Under California law, a licensed residential facility serving six or fewer residents is treated as a regular residential use of property — allowed in any residential zone, just like a family home, and it can't be required to get a special use permit that single-family homes don't need.
- **A family home is just a home.** A certified family home (Path A) is likewise just a home. For larger facilities (7+), check local zoning.
- **Fair Housing protects you.** Either way, homes for people with disabilities are protected under the federal Fair Housing Act — confirm specifics with your city or county.

Plan resident type and capacity before you buy. The single most expensive mistake is signing on a property before you confirm the fire and building requirements for your intended capacity and resident type. A 6-or-fewer ambulatory home is treated much like a house; a larger home, or one serving nonambulatory residents, can trigger sprinklers and construction upgrades. Confirm with your local fire authority and Community Care Licensing first — it's far cheaper to choose the right property than to retrofit the wrong one.

PART 8

Who You Actually Need — Staffing

In short: *In a certified family home (Path A), you and your household are the support, with the FHA's professional oversight. In a licensed ARF (Path B), plan for a certified administrator, direct-care staff (DSPs), nurse/consultant support at higher levels, and billing — one person can wear several hats at the start. Staffing scales with your service level.*

8.1 The roles

Certified family home (Path A): you and your household are the support. The FHA provides the professional oversight, training, and monitoring, and helps arrange respite/relief — you don't build a staff. Licensed ARF (Path B): plan for a real team.

- **Certified Administrator:** runs the facility and holds the certificate; the license lives with this role.
- **Direct-care staff (DSPs):** deliver day-to-day care and training; must complete DSP training and be awake at night at most service levels (with limited exceptions at Levels 2 and 3).
- **Registered Nurse / consultants:** for higher service levels and residents with health or behavioral needs — a Level 4 home and above brings more consultant and staffing hours.
- **Administrative / billing support:** handles regional-center billing and documentation.

Staffing scales with the service level. Staffing hours rise sharply with your service level — for example, a Level 2 four-bed home is modeled around 168 staff hours per week, and hours climb at higher levels. At most service levels, staff must be awake at all times, including overnight. Because staffing is the biggest cost and the biggest driver of your rate, model your staff schedule to your service level before you open.

8.2 Recruiting and keeping DSPs — the real make-or-break

Direct support professionals are the heart of an ARF and the hardest thing to keep. DSP turnover is high across the whole field, and a home that can't reliably staff its shifts can't safely support residents or keep its authorizations — so treat recruiting and retention as your core operation, not an afterthought.

- **Recruit continuously and hire for heart:** specific tasks can be trained; patience, reliability, and genuine care for people with disabilities cannot. Check references for dependability, not just experience.
- **Onboard to the standard:** real orientation, the required DSP training and live-scan background checks, and a competency check before a DSP supports someone alone — this protects residents and satisfies CCL and the regional center.
- **Retain deliberately:** consistent schedules, correct and on-time pay, real support when a DSP hits a hard day, and recognition. Every DSP who quits costs you re-hiring, re-training, and often continuity for the resident they supported — turnover is the biggest hidden cost in residential care.

PART 9

Policies, Records, Resident Rights & The Person's Journey

In short: *Your policies and documentation are what the regional center (and CCL, for an ARF) checks and what you're paid against. Build them to the standard from day one. This Part also walks how a person actually gets placed with you — the step you can't get paid without.*

9.1 Policies & records

You keep clear documentation covering medications, emergencies, incident reporting, and the rights of the person you support — and the Individual Program Plan (IPP) drives what you deliver and record. Even in a family home, you keep this documentation; in an ARF, Title 22 and your vendorization require it. This documentation is both the compliance requirement and the backbone of a safe, well-run operation. Build a simple system that keeps each person's plan, service records, and incident documentation current, because the regional center reviews it and CCL can inspect it.

9.2 Documentation & staying inspection-ready

Whatever your state requires, the agencies that pass surveys and payer audits cleanly all do the same thing: they keep complete, current files from day one instead of scrambling the week before an inspection. Build these habits now, because reconstructing records after the fact is nearly impossible:

- **A file for every caregiver:** the application, background checks, training and competency records, health/TB screening, and a signed acknowledgment of your policies — all dated and kept current.
- **A file for every client:** the assessment, the service/care plan, the signed service agreement, consents, ongoing visit notes, and any payer authorizations.
- **A visit record for every shift:** who worked, when they arrived and left, and what care was provided — and, for Medicaid, the matching EVV record.
- **An incident and complaint log:** every incident, injury, and complaint and exactly how you resolved it — inspectors look for a real, used quality-improvement process, not a binder that's never been opened.

9.3 The person's journey — eligibility to move-in

You can't get paid until the regional center places someone with you and their services are authorized. Here's how that happens:

1. **The person is a regional center client.** They've been found eligible under the Lanterman Act and are served by a regional center, which assigns a service coordinator.
2. **The IPP calls for your service.** The person's Individual Program Plan identifies a need for a family home or a residential facility at a certain service level.
3. **You're vendored and visible.** Once you're vendored (and licensed, for an ARF), the regional center's service coordinators can refer clients to you — remember, vendorization doesn't guarantee referrals, so let coordinators know your openings and specialty.
4. **The match and placement.** The service coordinator, the person (and family), and you confirm the fit; for a family home the FHA leads the match. The person moves in.
5. **Services begin.** With the placement in the IPP and authorized, you're delivering a billable service.

Your census is a relationships game. *Because service coordinators refer clients and vendorization doesn't guarantee referrals, your census depends on relationships and reputation, not marketing. Get known to your regional center and its service coordinators as the provider who supports people well, communicates, and handles a placement smoothly — and stay in regular, helpful contact so that when a coordinator has a client who needs your service level, you're the name they think of first. Two practical moves help: keep your openings and specialty in front of the coordinators, and start on Path A if you can so you have revenue and a track record while you build.*

PART 10

How You Get Paid

In short: Vendorization unlocks payment. For an ARF, residential services are billed monthly, per resident, at your vendored rate — set by your service level from the DDS rate models. Room and board is handled separately. For a family home, the FHA bills and pays you.

10.1 How much — and why it varies

- **The rate model:** California reformed residential rates effective January 1, 2025. Rates now come from standardized rate models (the DDS Vendor Rate Study), built for each service and service level, and published per regional center — so a Level 3 home in San Diego may differ from Los Angeles.
- **Service level drives it:** Levels 2–7 reflect residents' needs and required staffing; higher levels mean more staff and consultant hours and a higher rate. Level 7 is customized for extraordinary needs.
- **How it's paid:** ARF residential services are generally billed monthly, per resident, at your vendored rate. Room and board is handled separately from the service rate.
- **Where to look:** find your exact figure in your regional center's published rate models (DDS → Vendor Rates), or ask the regional center — rates update, so confirm the current number before you budget.

10.2 The money math — how a group home pencils out

ARF economics are different from an hourly private-pay business — your revenue is the vendored rate per resident per month, and your biggest cost is direct-care labor. Lay it out with your own numbers:

- **Revenue:** your vendored monthly rate multiplied by your authorized residents — set by your service level (2–7) from the DDS rate models and published per regional center.
- **Cost:** DSP wages and payroll taxes are the bulk; then RN/consultant hours at higher levels, administration, insurance, and the building. Staffing scales with your service level (a Level 2 four-bed home is modeled around 168 staff hours a week).
- **The lever:** because the rate is set by the model and your service level, your margin comes from staffing efficiently to your authorized level and keeping your beds filled with authorized residents — utilization, not rate, is the number you manage.
- **Room and board stays separate:** the resident's rent and food are handled separately from the service rate (usually from their SSI/personal income) — keep that money and those agreements entirely separate from your regional-center billing.

10.3 Your first payment

1. **Confirm the authorization.** The person's placement and your service level/rate are authorized by the regional center.
2. **Deliver services and keep your documentation.**
3. **Bill the regional center monthly at your vendored rate.** The FHA does this for a family home; you do it directly for an ARF.
4. **Get paid — and accept the rate as payment in full.** You agree to accept the DDS-established rate as payment in full and to bill only for services actually provided; don't bill the person for a covered service. Keep your license, vendorization, and authorizations current so payment never lapses.

PART 11

What It Costs, How Long It Takes & Who to Call

In short: A family home (Path A) can move relatively quickly; a licensed ARF (Path B) is a months-long project through administrator certification, licensing, and vendorization. Your regional center is the center of gravity, with Community Care Licensing and your local fire authority close behind. Here's the list and the scripts.

11.1 Timeline & startup costs

A certified family home (Path A) can move relatively quickly — it's driven by the FHA's approval and a good match. A licensed ARF (Path B) is a months-long project: administrator certification, securing the property, the Community Care Licensing application and inspection, and vendorization (a 45-day decision once your application is complete) — often several months end to end. Family-home costs are modest — mainly your time, screening, and getting your home ready. An ARF adds real costs: the administrator certification course and exam, business formation and insurance, the property itself (buy or lease), fire-clearance and licensing requirements, staff payroll, and your policies and procedures. Confirm current fees and timelines with CCL and your regional center.

11.2 The statewide offices (same for everyone)

For...	Contact
General DDS info & the regional center lookup	DDS — 833-421-0061 · info@dds.ca.gov · dds.ca.gov/rc (enter your ZIP)
Vendorization (how to become a vendor)	dds.ca.gov → Vendor/Provider → Vendorization (DDS Provider Directory)
Licensing an ARF / group home	Community Care Licensing (CDSS) — Adult & Senior Care Program (Title 22)
Rates (the rate models)	dds.ca.gov → Vendor Rates (per-regional-center rate models)
Family Home Agency questions	DDS Office of Community Operations — ocofha@dds.ca.gov
Business, EIN & NPI	bizfileOnline.sos.ca.gov · irs.gov (EIN) · nppes.cms.hhs.gov (NPI)

11.3 The local calls — and exactly what to ask

① Your regional center — your most important call

"I want to open a family home / an ARF in this area. What's your vendorization process, and who is my resource coordinator?"

"What service levels and openings are most needed here right now?"

"(Family home) Which Family Home Agencies are vendored in this area?"

② Community Care Licensing (for an ARF) — the license

"I want to open an Adult Residential Facility for [capacity] adults. How do I start the license application, and when's the next orientation?"

"What are the fire-clearance and physical-plant requirements for my capacity and resident type (ambulatory vs. nonambulatory)?"

③ Local fire authority — the fire clearance

"I'm licensing an ARF for [capacity] residents, [ambulatory / some nonambulatory]. What fire clearance do I need, and do I need sprinklers?"

"Who performs the fire-clearance inspection, and how do I schedule it?"

11.4 Worked examples — California's two biggest markets

Most people start in or near one of these two. Note a real California quirk: Los Angeles County is split among several regional centers by health district, while San Diego has one.

- **Los Angeles (find your regional center by ZIP):** LA County is served by several regional centers — you're assigned by your home's ZIP/health district (use the DDS lookup at dds.ca.gov/rc). The main LA-area regional centers: Frank D. Lanterman (213) 383-1300 (vendorization@lanterman.org); Harbor (310) 540-1711; North LA County (818) 778-1900; San Gabriel/Pomona (909) 620-7722; South Central LA (213) 744-7000; Westside (310)

258-4000; plus Eastern LA. Licensing: Community Care Licensing (CDSS) regional office for LA; local fire authority (city fire department or LA County Fire) for the fire clearance. The quirk: don't guess your regional center — LA is carved into health districts, so confirm yours by ZIP first.

- **San Diego (San Diego Regional Center):** One clean regional center — San Diego Regional Center (SDRC), 4355 Ruffin Rd., Suite 200, San Diego, CA 92123; (858) 576-2931 (serves San Diego and Imperial counties). SDRC moved to the DDS Provider Directory for new vendorizations in 2026. Licensing: Community Care Licensing (CDSS) San Diego regional office; San Diego (city or county) fire authority for the fire clearance. The quirk: San Diego uses one regional center for the whole county, which makes it simpler than LA — attend SDRC's provider information session before you apply.

11.5 How to find your own regional center & offices

1. **Enter your home's ZIP at dds.ca.gov/rc.** This finds your regional center — it's assigned by location.
2. **Call that regional center.** Ask for the vendorization / resource coordinator to start the process.
3. **For an ARF, contact CCL and your fire authority.** For the license, the Community Care Licensing regional office; for the fire clearance, your city or county fire authority.

Common questions, answered

Do I have to take Medicaid?

No. Many successful agencies run entirely on private pay, long-term-care insurance, and veterans' benefits — it's often the smarter way to start, because you earn immediately without the enrollment, contracting, and EVV that Medicaid requires. You can add Medicaid later once you have cash flow and a team.

Can I run this from home at first?

Often yes for a small non-medical agency — the care happens in the client's home, so you may not need a storefront to start. Confirm your local zoning and your license/registration rules (some require a business address), and know you'll want real office space as you grow.

Should my caregivers be employees or independent contractors?

Almost always W-2 employees. Classifying caregivers as 1099 contractors to save on taxes is one of the most common — and most expensive — mistakes in home care; it invites wage-and-hour and tax liability, and most payers and insurers require employees. Treat them as employees and carry workers' comp.

How much cash do I really need to start?

Enough to cover several weeks — ideally a couple of months — of caregiver payroll before your first payer reimbursements arrive, plus your license/insurance/software setup. Under-capitalization is the number-one reason new agencies fail; line up a business line of credit before you scale.

How long until it's profitable?

Most agencies take several months to a year to build a steady client base and referral flow. Starting on private pay shortens the runway because you're paid quickly; Medicaid volume comes later and pays slower. Plan your finances for a ramp, not an overnight full census.

Do I need experience in health care?

It helps, but plenty of successful owners come from business, sales, or family-caregiving backgrounds and hire the clinical expertise they need (for example, a nurse where the state requires one). What matters more is being organized, responsive, good with people, and disciplined about compliance and cash.

What actually makes one agency win over another?

Reliability. Families and referral sources choose — and stay with — the agency that answers the phone, starts care fast, sends the same dependable caregiver, communicates, and never leaves a shift unstaffed. Get that right and referrals compound; get it wrong and no amount of marketing saves you.

Do I have to open a licensed facility, or can I start smaller?

You can start smaller — Path A lets you become a certified family home through a Family Home Agency, with an adult living in your home and no CCL license. It's the fastest way in and the best way to learn the regional center system before you take on a licensed ARF (Path B). Many providers grow from A to B.

Why are there two approvals for an ARF — a license AND a vendorization?

Because they do different things. Community Care Licensing licenses the FACILITY under Title 22 (health, safety, physical plant), while your regional center VENDORS you (Title 17) and assigns your service level and rate so you can be paid. They're separate applications — run them in parallel, and keep your legal name, EIN, and NPI identical across both.

Does the regional center pay for the person's rent and food?

No — the regional center pays for the SERVICE (the support and supervision you provide), not room and board, which is handled separately (usually from the person's SSI/personal income). Keep the two completely separate in your agreements and your books; mixing them is a common and serious mistake.

Do I really need experience to open an ARF?

Regional centers expect the administrator to have direct experience in a vendored residential setting, and the administrator must be certified. Many people work as a DSP in an ARF first to build that experience — it's also how you'll actually know how to run the home safely and pass your reviews.

You can do this

Opening a group home business in California is real work — the regional center system has a lot of moving parts — but it's a real, walkable path, and people across the state do it every day, giving someone a real home in the community. You don't have to be a lawyer or hire a consultant. Whether you start small with a certified family home or open a licensed facility, take it one step at a time: set up the business, learn the standards, get certified/licensed and vendored, ready the home, and build the relationships that bring you placements. Verify the current details with the State as you go, and lean on your regional center — that's what they're there for.

***Reminder.** Prepared for PolicyReady.org and grounded in California DDS and Community Care Licensing rules, the Lanterman Act, and Title 17 and Title 22 of the California Code of Regulations as of 2026. This is general information, not legal, tax, or billing advice. Requirements, rates, and codes change — always verify current details with the State of California before you act.*